

STRIVE INDEPENDENCE CLINICAL HANDBOOK

Medication Management Handbook

A Progressive Activity Guide for Therapists, Caregivers & Families

EASY Activities 1–3	MODERATE Activities 4–6	HARD Activities 7–9	COMPLEX Activities 10–12
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12 Progressive Activities · 4 Difficulty Levels · Full Therapist Guidance
Executive Function · Memory · Attention · Safety Judgment · Problem-Solving

STRIVE INDEPENDENCE
Occupational Therapy · Speech-Language Pathology · Cognitive Rehabilitation

Quick Administration Guide

A fast visual reference for choosing, running, and documenting medication management activities.

EASY Activities 1-3	MODERATE Activities 4-6	HARD Activities 7-9	COMPLEX Activities 10-12
Read labels, match time slots, identify basic safety.	Plan routines, calculate refills, update medication lists.	Detect errors, manage multi-day schedules, resist unsafe choices.	Simulate real-world demands with distractions and decision-making.

Session Flow

1. Choose	2. Set Up	3. Coach	4. Document
Pick the level based on current cognition, safety awareness, and caregiver goals.	Use the materials list first. Keep only the needed bottles, schedule, or organizer visible.	Ask guided questions before correcting. Use the exact Say/Tip prompts when possible.	Record accuracy, cues, errors, safety concerns, and next recommended activity.

Quick Documentation Prompts

Accuracy	Correct placements / responses, missed items, self-corrections.
Cueing	Independent, verbal cue, visual cue, model, hand-over-hand, caregiver-assisted.
Errors	Skipped label, guessed by color, missed empty slot, doubled dose, poor problem solving.
Safety	Recognized unsafe behavior, asked for help, used pharmacy/doctor as resource.
Next Step	Repeat, simplify, progress to next activity, or add caregiver training/home carryover.

WELCOME

How to Use This Handbook

This handbook is designed for occupational therapists, speech-language pathologists, rehabilitation aides, and family caregivers working with individuals recovering from stroke, traumatic brain injury, or other conditions affecting cognitive function.

Each activity page gives you everything you need to run a session — no preparation required. Just open to the page, read along, and follow the steps.

The Structure of Each Activity Page

Overview A plain-language description of the task and why it matters clinically.	What You Will Need Materials and setup checklist before the session begins.
Step-by-Step Guide Numbered steps with what to do, what to say, and clinical tips.	Answer Key Correct responses and scoring guidance for the therapist.
What to Watch For Common errors and what they indicate clinically.	If They Struggle / Succeed Prompts for guiding errors, and next steps after success.

Coaching Philosophy

This handbook follows a guided discovery approach — rather than giving the patient the correct answer, you ask questions that lead them to find it. The goal is not accuracy on a worksheet; it is building durable real-world habits.

→ **KEY CONCEPT**

When a patient makes an error, always ask "What does the label say?" before correcting them. Self-correction is far more powerful than being told the answer.

★ **CLINICAL TIP**

If a patient seems overwhelmed, stop and do just one step. Partial completion with good understanding is more valuable than rushing through all steps with poor retention.

Difficulty Levels at a Glance

Level	Activities	Cognitive Demands	Ideal For
Easy	1 – 3	Reading, sorting, basic judgment	Early recovery, cognitive screening, orientation sessions

Moderate	4 – 6	Planning, calculation, updating information	Patients progressing toward discharge planning
Hard	7 – 9	Error detection, multi-day scheduling, impulse control	Outpatient rehab, discharge readiness
Complex	10 – 12	Executive function, divided attention, multi-step simulation	Community reintegration, independent living planning

Important Notes for Family Members

You do not need a clinical background to use this handbook. The step-by-step format is written in plain language. Your role is to follow the steps, ask the suggested questions, and encourage your family member — not to grade them.

! SAFETY NOTE

This handbook does not replace a licensed therapist's clinical judgment. Activities should be selected and supervised by a qualified healthcare professional whenever possible. Do not use this handbook to make decisions about a patient's medication regimen.

Session Log

Date	Activity #	Completed?	Notes / Observations
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	
		☐ Yes ☐ No	

SECTION 1

Easy

Activities 1 – 3

ACTIVITY

1

EASY LEVEL · Cognitive Rehabilitation

Match the Medication to the Time of Day

Overview

The patient looks at four medication bottles and reads the label on each one, then matches each medication to the correct time slot in the daily schedule — Morning, Lunch, Dinner, or Bedtime. This builds the foundational habit of reading labels rather than relying on guesswork or memory.

WHAT YOU WILL NEED

- App open to Activity 1 — or this printed page
- A quiet room with no distractions
- 5 to 10 minutes

THE MEDICATION SCHEDULE — READ THIS TO THE PATIENT

Medication	What the Label Says
Metformin	Take 1 tablet in the morning
Vitamin D	Take 1 capsule at lunch
Atorvastatin	Take 1 tablet at bedtime
Aspirin 81 mg	Take 1 tablet in the morning

STEP-BY-STEP GUIDE FOR THE HELPER

1

Orient the patient

Do: Point to the four time boxes: Morning, Lunch, Dinner, Bedtime. Explain that each medication belongs in one box.

Say: "We're going to match each medication to the time of day it should be taken. Read the label — then drag it into the right box."

Tip: Sit beside the patient so you can both see the screen at the same time.

2

Work through medications one at a time

Do: Ask the patient to read each label and place each medication before moving to the next.

Say: "Start with the first bottle. Read me the label. Now — where does it go?"

Tip: One bottle at a time. Do not hand over all four at once.

3

Let them self-check

Do: Once all four are placed, ask the patient to read each one back before you check.

Say: *"Before we look at the answers — read each slot back to me and tell me why you placed it there."*

Tip: Self-checking is a clinical skill in itself. Do not skip this step.

4

Review and discuss

Do: Confirm correct answers. For any wrong ones, point to the label — do not just say the correct time.

Say: *"Let's look at the Atorvastatin — read the very last line of the label. Where does that put it?"*

Tip: Never say 'wrong.' Say 'let's double-check that one together.'

ANSWER KEY

Time Slot	Correct Medication(s)
Morning	Metformin + Aspirin 81 mg (two medications)
Lunch	Vitamin D
Dinner	Empty — nothing goes here
Bedtime	Atorvastatin

WHAT TO WATCH FOR

- Patient puts Atorvastatin in morning — most common error
- Only one medication placed in morning slot (missing the second)
- Patient selects medication by pill color rather than reading the label
- Patient leaves the Dinner slot without confirming it should be empty

IF THEY STRUGGLE

- Point to the label: "Read the last line on this bottle. What time does it say?"
- For double morning: "Count how many labels say 'morning.' Let's count together."
- For color guessing: "Ignore the colors. Just read the words on the label."

✓ IF THEY SUCCEED

Say: *"Great work — you read every label before placing anything. That is exactly the habit that keeps you safe at home." Move on to Activity 2.*

ACTIVITY

2

EASY LEVEL · Cognitive Rehabilitation

Fill a One-Day Pill Organizer

Overview

The patient fills a four-compartment pill organizer — Morning, Afternoon, Evening, Bedtime — using three medications and their written directions. Some compartments will stay empty. This builds the daily routine of reading labels before placing pills, and reinforces that not every slot needs a medication.

WHAT YOU WILL NEED

- App open to Activity 2 — or a real 4-slot pill organizer
- Three medication bottles or printed labels (listed below)
- Pen and paper if completing on paper
- About 10 minutes

MEDICATION LABELS — READ EACH ONE ALOUD

Medication	Label Instruction
Lisinopril 10 mg	Take 1 tablet in the morning
Calcium Carbonate 500 mg	Take 1 tablet in the evening
Melatonin 5 mg	Take 1 tablet at bedtime

STEP-BY-STEP GUIDE FOR THE HELPER

1

Show the pill organizer

Do: Point to each of the four compartments and read their labels aloud: Morning, Afternoon, Evening, Bedtime.

Say: "This is a pill organizer. Each section is for a different time of day. We're going to put the right medications in the right sections."

Tip: If using a real pill organizer, place it between you on the table.

2

Introduce medications one at a time

Do: Show the first bottle. Ask the patient to read the label out loud.

Say: "Pick up the first bottle. Read me the name and what it says to do."

Tip: If reading is difficult, read the label yourself and ask the patient to point to the right compartment.

3

Place, then continue

Do: After the patient places each medication, confirm and move to the next bottle.

Say: "Good. Now the next one — read the label first, then decide where it goes."

Tip: Pause after each placement. Ask: 'Are you sure? Read the label one more time.'

4

Address the empty slot

Do: After all medications are placed, ask about the Afternoon slot.

Say: "Is there anything in the Afternoon slot? Why is that okay?"

Tip: The correct answer: no medication is scheduled for afternoon. Confirm that an empty slot is not an error.

5

Teach-back

Do: Ask the patient to explain the completed organizer back to you in their own words.

Say: "Pretend I've never seen this before. Walk me through your whole pill organizer — why is each one where it is?"

Tip: Teach-back is the gold standard for confirming genuine understanding versus surface compliance.

ANSWER KEY

Compartment	What Goes There
Morning	Lisinopril 10 mg
Afternoon	Empty — no medication scheduled
Evening	Calcium Carbonate 500 mg
Bedtime	Melatonin 5 mg

WHAT TO WATCH FOR

- Melatonin placed in Evening instead of Bedtime — 'evening' and 'bedtime' feel the same to many patients
- Something placed in the Afternoon slot when no medication belongs there
- Patient picks up a pill by color before reading the label
- Patient rushes through without reading each label individually

IF THEY STRUGGLE

- Melatonin in wrong slot: "Read the label again — does it say 'evening' or 'bedtime'? Point to the exact word."
- Afternoon slot filled incorrectly: "Let's read every label — does any one of them say 'afternoon'?"
- Choosing by color: "Let's pretend all the pills are the same color. Now read the label — where does it go?"

✓ IF THEY SUCCEED

Say: "You just filled a real pill organizer correctly by reading every single label. This is exactly what you'll do at home each week." Move on to Activity 3.

ACTIVITY

3

EASY LEVEL · Cognitive Rehabilitation

Identify Medication Safety Mistakes

Overview

The patient hears a series of short statements about common medication behaviors and decides whether each one is Safe or Unsafe. No pharmacology knowledge is needed — this is a judgment and reasoning activity that builds foundational health literacy and safety awareness.

WHAT YOU WILL NEED

- App open to Activity 3 — or this printed page
- Read each statement aloud; patient responds verbally or by pointing
- About 10 to 15 minutes

STATEMENTS — READ ALOUD AND ASK: SAFE OR UNSAFE?

Statement	Correct Answer
Take medicine only as prescribed by your doctor.	✓ Safe
Skip your blood pressure pill if you feel fine today.	✗ Unsafe
Use your neighbor's medication for the same symptoms.	✗ Unsafe
Call the pharmacy if you are unsure about the instructions.	✓ Safe
Take double the dose to make up for a missed one.	✗ Unsafe
Store medications away from children and heat.	✓ Safe
Stop your antibiotic early because you feel better.	✗ Unsafe
Keep a written list of all medications to share with doctors.	✓ Safe

STEP-BY-STEP GUIDE FOR THE HELPER

1

Explain the task simply

Do: Tell the patient they will hear a statement and just need to say Safe or Unsafe.

Say: "I'm going to read you some things people do with their medications. After each one — safe or unsafe? Go with your gut."

Tip: Demonstrate first: 'Someone takes their medication every day as directed. Safe or unsafe?' Let them answer.

2

Read each statement slowly

Do: Read one statement at a time. Pause for 10 to 15 seconds before prompting.

Say: "Ready? First one: [read statement]. Safe or unsafe?"

Tip: Do not rush. Give the patient time to think before you repeat or prompt.

3

Respond to each answer

Do: After each response, confirm or gently redirect using the explanations below.

Say: "(If correct) That's right. (If incorrect) Let's think about that — what might happen if someone did that?"

Tip: Never just say 'wrong.' Ask them to reason through the consequences first.

4

Return to misses at the end

Do: After all eight statements, revisit any incorrect answers.

Say: "Let's look at the ones we want to go over again. Why do you think skipping a blood pressure pill could be unsafe even if you feel fine?"

Tip: Use the table below to guide the discussion — your goal is reasoning, not lecturing.

ANSWER KEY

Unsafe Statement	Why It Is Dangerous
Skipping blood pressure medication	Blood pressure can be dangerously high with zero symptoms. Skipping creates real risk on 'good' days.
Using someone else's medication	Illegal and medically dangerous — doses, allergies, and drug interactions differ between people.
Double-dosing to catch up	Can cause overdose or serious side effects. Always call the pharmacist first.
Stopping antibiotics early	Bacteria survive and return stronger. The full course must be completed.

WHAT TO WATCH FOR

- Patient says blood pressure skipping is safe — a very common real-world belief
- Patient thinks stopping antibiotics early is fine when feeling better
- Patient answers quickly without reasoning — prompt them to slow down and explain
- Patient says double-dosing 'probably isn't a big deal'

IF THEY STRUGGLE

→ Blood pressure: "What if your blood pressure is high but there's no headache? How would you know?"

→ Double dose: "What does your body experience when it suddenly gets twice the medication at once?"

→ Antibiotic: "What do you think happens to the bacteria that are still there when you stop early?"

✓ IF THEY SUCCEED

Say: "You just reviewed eight medication safety rules — the same ones that protect you every day at home. Well done." Move on to Activity 4.

SECTION 2

Moderate

Activities 4 – 6

ACTIVITY

4

MODERATE LEVEL · Cognitive Rehabilitation

Build a Morning and Evening Medication Routine

Overview

The patient organizes five medications into a structured daily routine split into morning and evening windows. The key challenge is a medication labeled 'every 12 hours' — requiring the patient to recognize that it belongs in both columns. This teaches interpretation of frequency language, not just copying instructions literally.

WHAT YOU WILL NEED

- App open to Activity 4 — or a blank two-column schedule (Morning / Evening)
- The 5 medication labels listed below
- Pencil if completing on paper
- About 15 minutes

THE 5 MEDICATIONS — READ EACH ONE ALOUD BEFORE STARTING

Medication	Label Instruction
Blood Pressure Pill (Amlodipine)	Take once daily in the morning
Cholesterol Pill (Atorvastatin)	Take once daily at bedtime
Antibiotic (Amoxicillin)	Take every 12 hours
Multivitamin	Take once daily with breakfast
Eye Drops (Timolol)	Use every evening

STEP-BY-STEP GUIDE FOR THE HELPER

1

Set up the two columns

Do: Draw or show Morning and Evening/Bedtime columns. Explain that some medications go in one, some in both.

Say: "We're building your daily medication schedule. Some medications go in the morning, some in the evening — and one might go in both. Read each label carefully."

Tip: Write the column headers large if using paper.

2

One medication at a time

Do: Point to or hand the first medication. Ask the patient to read the label and choose a column.

Say: "Start with the blood pressure pill. Read the label — which column?"

Tip: Do not give all five at once. One at a time keeps attention focused.

3

Pause on 'every 12 hours'

Do: When you reach Amoxicillin, pause. Ask the patient what 'every 12 hours' means before placing it.

Say: "This one says 'every 12 hours.' How many times a day is that? So — which columns does it go in?"

Tip: If stuck: 'If you take it at 8 AM, when is 12 hours later? So when is the second dose?'

4

Review the completed schedule

Do: Once all five are placed, read the full schedule back out loud together.

Say: "Let's read your schedule. Morning: you have... Evening: you have... Does anything look missing or off?"

Tip: Ask the patient to self-check before you point out any errors.

5

Teach-back

Do: Ask the patient to explain the schedule in their own words.

Say: "Explain your daily routine to me like I've never heard it before."

Tip: If they can explain it clearly, they understand it. If not, revisit the specific label that is unclear.

ANSWER KEY

Time	Medications
Morning	Blood Pressure Pill · Amoxicillin (Dose 1) · Multivitamin
Evening / Bedtime	Amoxicillin (Dose 2) · Cholesterol Pill · Eye Drops

WHAT TO WATCH FOR

- Antibiotic placed in only one column — the main challenge of this activity
- Cholesterol pill placed in morning instead of bedtime
- Multivitamin missed because 'with breakfast' doesn't sound like a time slot
- Eye drops missed or placed in morning

IF THEY STRUGGLE

- Antibiotic in one column only: "Every 12 hours means twice a day. If morning is Dose 1 — when is Dose 2?"
- Cholesterol in morning: "Read the label again — does it say morning or bedtime?"
- Missing multivitamin: "What happens at breakfast? Is that morning or a different time of day?"

✓ IF THEY SUCCEED

Say: "You built a complete daily routine — and you figured out the 12-hour medication. That is one of the

hardest parts, and you got it." Move on to Activity 5.

ACTIVITY

5

MODERATE LEVEL · Cognitive Rehabilitation

Refill Reminder Task

Overview

The patient reads a medication bottle showing how many pills remain and how often to take them, then calculates how many days of medication are left. Based on that, they choose the safest action. This builds the real-world habit of calling the pharmacy before running out — not after.

WHAT YOU WILL NEED

- App open to Activity 5 — or this printed page
- Pencil and paper for the calculation (optional)
- About 10 to 15 minutes

THE MEDICATION BOTTLE — READ THIS ALOUD

Item	Information
Medication	Gabapentin 300 mg
Pills remaining	12 pills
Directions	Take 1 pill, 3 times per day
Today's date	June 10
Pharmacy refill number	(555) 842-0011

PATIENT RESPONSE OPTIONS

- A Wait until the bottle is completely empty, then call.
- B Call the pharmacy today or tomorrow to request a refill. ✓ Correct Answer
- C Take fewer pills each day to make the supply last longer.
- D Stop the medication since you are almost out anyway.

STEP-BY-STEP GUIDE FOR THE HELPER

- 1 Read the bottle together
- Do: Read each line of the bottle information below aloud to the patient. Point to each number as you say

2

it.

Say: "Let's read this bottle together. It says 12 pills are left. Take 1 pill, 3 times a day. Today is June 10."

Tip: If the patient has trouble reading, read it yourself and ask them to repeat each fact back to you.

3

Ask about days remaining

Do: Ask the patient to figure out how many days of medication are left. Paper and pencil are fine.

Say: "If you take 3 pills every day and you have 12 left — how many days will they last?"

Tip: If stuck: 'Day 1 uses 3 pills. Day 2 uses 3 more. Now how many days total?'

4

Confirm the math together

Do: Walk through the calculation: $12 \div 3 = 4$ days. Name the date the pills run out.

Say: "12 pills divided by 3 per day equals 4 days. So the pills run out on June 14. What day is that from today?"

Tip: Use a calendar or count on fingers if needed. The goal is understanding, not arithmetic speed.

5

Present the four options

Do: Read the four options below one at a time. Let the patient decide without hinting.

Say: "Now that you know you only have 4 days left — what's the smartest thing to do?"

Tip: Cover other options while reading each one if attention is limited.

6

Ask them to explain the answer

Do: After they choose, ask why that is the best choice.

Say: "Good choice. Tell me — why would waiting until the bottle is empty be a problem?"

Tip: If they chose wrong: 'What if the pharmacy needs 3 days to fill it and you only have 2 days left?'

ANSWER KEY

Calculation Step	Answer
Pills per day	1 pill $\times$ 3 times = 3 pills per day
Days remaining	12 pills $\div$ 3 per day = 4 days
Date pills run out	June 10 + 4 days = June 14
Best action	Call the pharmacy today or tomorrow (Option B)

WHAT TO WATCH FOR

- Patient chooses Option A — waiting until empty, which risks a gap if the pharmacy has delays
- Patient chooses Option C — reducing a prescribed dose is never safe without provider approval
- Patient makes a calculation error — focus on the concept (plan ahead), not the arithmetic

IF THEY STRUGGLE

- Option A chosen: "What if the pharmacy needs 3 days to fill it and you have 2 days left?"
- Option C chosen: "Changing your dose without the doctor's approval can be dangerous. What should you do instead?"
- Option D chosen: "What happens to your body if you stop a medication like Gabapentin suddenly?"

✓ IF THEY SUCCEED

Say: "You figured out exactly when to call for a refill — before you run out. That habit prevents missed doses. Well done." Move on to Activity 6.

ACTIVITY

6

MODERATE LEVEL · Cognitive Rehabilitation

Medication List Update

Overview

The patient has an old medication list and a written note from their doctor with changes. They must apply the doctor's instructions — adding a new medication, removing a stopped one, and updating a frequency — to produce a correct updated list. This mirrors the real task of maintaining an accurate medication record after every appointment.

WHAT YOU WILL NEED

- App open to Activity 6 — or printed copies of both documents below
- A pen to mark changes if using paper
- About 15 minutes

DOCUMENT 1 — OLD MEDICATION LIST

Medication	Current Instructions
Metformin 500 mg	Take once daily
Lisinopril 10 mg	Take once daily
Aspirin 81 mg	Take once daily

DOCTOR'S NOTE — READ THIS ALOUD SLOWLY

1. Metformin — INCREASE to twice daily (same tablet, just more often)
2. Aspirin 81 mg — STOP. Do not take anymore.
3. Lisinopril 10 mg — CONTINUE. No changes.
4. Vitamin B12 1000 mcg — NEW. Add to list, take once daily.

STEP-BY-STEP GUIDE FOR THE HELPER

1

Read both documents together

Do: Read Document 1 (old list) aloud. Then read the Doctor's Note below aloud, slowly and clearly.

Say: "This is your old medication list. This is what your doctor told you to change. We're going to update the list using the note."

Tip: Point to each item in the doctor's note as you read it. Use a finger to track.

2

Start with what stays the same

Do: Ask the patient to identify which medication the doctor said to continue unchanged.

Say: *"Look at the doctor's note. Which medication stays exactly the same? Find it and circle it."*

Tip: Starting with the 'no change' item builds confidence before the harder changes.

3

Find the medication to stop

Do: Ask the patient to find and cross off the discontinued medication.

Say: *"The doctor said to STOP one medication. Which one? Cross it off the list."*

Tip: Watch carefully — they should cross off Aspirin, not Lisinopril.

4

Apply the frequency change

Do: Ask the patient to update Metformin's frequency on the list.

Say: *"The doctor changed how often you take Metformin. How often was it? How often is it now? Update the list."*

Tip: Common error: changing the dose amount instead of the frequency. Watch for this.

5

Add the new medication

Do: Ask the patient to write Vitamin B12 at the bottom of the updated list.

Say: *"The doctor added a brand new medication. Which one? Write it on the list with its directions."*

Tip: Ask: 'Is there anything the doctor ADDED that is not on our list yet?' — this step is commonly missed.

6

Read the final list back

Do: Have the patient read the completed updated list aloud.

Say: *"Read me the final list. Every medication, the dose, and how often."*

Tip: Compare to the Answer Key. Confirm all four changes were made correctly.

ANSWER KEY

Medication	Updated Instructions	What Changed
Metformin 500 mg	Take TWICE daily	Frequency increased
Lisinopril 10 mg	Take once daily	No change
Vitamin B12 1000 mcg	Take once daily	Newly added
Aspirin 81 mg	REMOVED from list	Discontinued

WHAT TO WATCH FOR

- Aspirin still on the list — the most common error (failure to inhibit old information)
- Vitamin B12 not added — commonly missed because it was not on the original list
- Metformin dose amount changed instead of frequency

- Lisinopril changed even though the note says 'no change'

IF THEY STRUGGLE

- Aspirin still on list: "Read the doctor's note again — which medication did it say to STOP?"
- B12 missing: "Is there anything the doctor said to ADD that is not on our updated list?"
- Wrong Metformin change: "The tablet is still 500 mg. What exactly changed? Read that line again."

✓ IF THEY SUCCEED

Say: "You just did exactly what you'll need to do every time your doctor makes a change. Keeping an accurate medication list could prevent a serious error." Move on to Activity 7.

SECTION 3

Hard Problem-Solving & Safety

Activities 7 – 9

ACTIVITY

7

HARD LEVEL · Cognitive Rehabilitation

Resolve Conflicting Medication Instructions

Overview

The patient is shown two documents about the same medication that contain conflicting dosing instructions. One says once daily; the other says twice daily. They must recognize the conflict and identify the only safe response: stop and call before taking the next dose. This activity directly addresses one of the highest-risk moments in medication management — transitions between care settings.

WHAT YOU WILL NEED

- App open to Activity 7 — or the two printed documents below
- About 15 to 20 minutes
- This activity should be followed by a discussion, not just a correct answer

PATIENT RESPONSE OPTIONS

- A Follow the prescription bottle — take once daily.
- B Follow the discharge paperwork — take twice daily.
- C Combine both instructions and take three tablets today to be safe.
- D Call the prescribing doctor or pharmacist to clarify BEFORE taking the next dose. ✓ Correct Answer

PRESCRIPTION BOTTLE Medication: Lisinopril 10 mg Instructions: Take 1 tablet ONCE daily	DISCHARGE PAPERWORK Medication: Lisinopril 10 mg Instructions: Take 1 tablet TWICE daily
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STEP-BY-STEP GUIDE FOR THE HELPER

- 1

Read both documents aloud
Do: Read the prescription bottle label, then the discharge document. Read them slowly, with a deliberate pause between.
Say: "I'm going to read you two documents about the same medication. Listen carefully."
Tip: Emphasize the words 'ONCE' and 'TWICE' when reading. Let the contrast land.
- 2

Ask if anything seems off

Do: Before presenting options, ask the patient whether they noticed anything unusual.

Say: "You just heard both documents. Does anything seem off to you? Do they say the same thing?"

Tip: Do not point out the conflict yet. Let the patient find it. If they miss it, ask: 'How many times a day does the bottle say? How many times does the paper say?'

3

Confirm the conflict is clear

Do: Once identified, name the conflict explicitly and clearly.

Say: "Right — the bottle says *ONCE* and the discharge paper says *TWICE*. They cannot both be right. So what should you do?"

Tip: Do not accept 'I'll just pick one.' Push for a safe action before presenting options.

4

Present the four options

Do: Read each option aloud. Let the patient decide. Do not hint.

Say: "Here are your choices. Think carefully about each one."

Tip: After they answer, ask why — even if they are correct. Reasoning is the goal.

5

Reinforce the principle

Do: After confirming the correct answer, name the universal rule.

Say: "The only safe answer when two instructions disagree is to stop and call. Not guess — call."

Tip: Ask: 'Who would you call?' Help them name a specific person — pharmacist, primary doctor, or discharge nurse.

ANSWER KEY

Option	Why It Is or Is Not Safe
A — Bottle	Still a guess. What if the bottle is outdated?
B — Discharge paper	Still a guess. What if there was a printing error?
C — Both combined	Dangerously wrong. Three tablets of Lisinopril in one day could be harmful.
D — Call first	Only safe option. Conflicting instructions must be resolved by a clinical provider.

WHAT TO WATCH FOR

- Patient picks A or B without recognizing that guessing is still unsafe
- Patient picks C — this is the most dangerous answer
- Patient says 'I'd just ask my daughter' — redirect to medical authority (pharmacist, doctor)
- Patient seems unconcerned by the conflict — this is a key teaching moment

IF THEY STRUGGLE

→ Picked A or B: "Even if you happened to pick the right one by luck — what if you're wrong? What happens then?"

- Picked C: "If one paper says once and another says twice, together that's three doses. Is that the right dose?"
- 'I'd just guess': "This is your blood pressure medication. What could happen if the dose is too high or too low?"

✓ IF THEY SUCCEED

Say: "This is one of the most important lessons in this entire handbook. When instructions don't match — stop and call. That one rule can prevent serious harm." Move on to Activity 8.

ACTIVITY

8

HARD LEVEL · Cognitive Rehabilitation

Create a Weekly Pillbox Schedule

Overview

The patient fills all fourteen compartments of a 7-day pill organizer (morning and evening slots) using four medications with different schedules. Some medications are taken every day, one only on specific days, and one only once per week in a higher quantity. This is one of the most demanding real-world medication tasks and tests sustained attention, planning, and memory across a full week.

WHAT YOU WILL NEED

- App open to Activity 8 — or a real 7-day pill organizer with morning and evening slots
- The medication directions below — read them aloud before starting
- The weekly answer grid below for checking work
- About 20 to 30 minutes — breaks are appropriate

THE 4 MEDICATIONS — READ EVERY LINE BEFORE STARTING

Medication	Frequency	Special Note
Medication A	1 pill every morning	Every day — 7 mornings
Medication B	1 pill every evening	Every day — 7 evenings
Medication C	1 pill on Mon, Wed, Fri mornings only	Only 3 days per week — skip other days
Medication D	2 pills on Sunday morning only	Once per week — 2 pills, Sunday only

STEP-BY-STEP GUIDE FOR THE HELPER

1

Review all medications first

Do: Read each medication and its frequency aloud. Ask the patient to repeat it back.

Say: "Before we fill anything — tell me when you take Medication C. Now tell me what's special about Medication D."

Tip: Spend extra time on C (three-day rule) and D (two pills, Sunday only). Errors here cascade across the whole week.

2

Start with Sunday

Do: Fill Sunday first. Name each medication the patient should place.

Say: "Let's start with Sunday. What goes in Sunday morning? Remember — this is the special day for

one medication."

Tip: Medication D goes here — two pills. Common error: placing only one pill, or placing it on other days.

3

Fill Monday and prompt for C

Do: Fill Monday. Prompt before filling: 'Is this a Medication C day?'

Say: "Monday — is this one of Medication C's days? What else goes in Monday morning?"

Tip: Monday gets A and C in the morning, B in the evening.

4

Continue through the week day by day

Do: For each remaining day, ask 'Is this a Medication C day?' before the patient fills it.

Say: "Wednesday — Medication C day or not? Thursday?"

Tip: Quick rule: Mon/Wed/Fri get C. Tue/Thu/Sat/Sun do not.

5

Self-check before reviewing

Do: Ask the patient to go through every compartment before you look at the answer key.

Say: "Before we check — look at every single slot. Does anything seem off?"

Tip: Self-monitoring is a core executive function skill. Never skip this step.

ANSWER KEY

Day	Morning	Evening
Sunday	A + D (2 pills)	B
Monday	A + C	B
Tuesday	A only	B
Wednesday	A + C	B
Thursday	A only	B
Friday	A + C	B
Saturday	A only	B

WHAT TO WATCH FOR

- Medication D placed on more than one day
- Only 1 pill of Medication D on Sunday instead of 2
- Medication C added on Tuesday, Thursday, or Saturday
- Medication C missing on Monday, Wednesday, or Friday
- Medication A or B missing from any day

IF THEY STRUGGLE

- D on multiple days: "Read the label — it says Sunday morning only. How many Sundays in a week?"
- Wrong count for D: "The label says 2 pills on Sunday. Count what you placed."
- C on wrong days: "Let's list the days for C: Monday, Wednesday, Friday. Is Thursday on that list?"

✓ IF THEY SUCCEED

Say: "You planned medications for an entire week — including ones that aren't taken every day. That is genuinely hard, and you did it. Excellent work." Move on to Activity 9.

ACTIVITY

9

HARD LEVEL · Cognitive Rehabilitation

Missed Dose Decision-Making

Overview

The patient works through two realistic missed-dose scenarios and must choose the safest course of action in each one. The central lesson is never double-dosing to 'catch up,' and always contacting a pharmacist or prescriber when unsure. These are the most common dangerous self-management errors in real-world medication management.

WHAT YOU WILL NEED

- App open to Activity 9 — or this printed page
- No additional materials needed
- About 15 to 20 minutes — this is discussion-based

PATIENT RESPONSE OPTIONS

- A Skip this dose entirely and wait until tomorrow morning.
- B Take the dose now — it has only been 2 hours.
- C Take two doses at noon to make up for missing this morning.
- D Check the medication instructions or call the pharmacist before deciding. ✓ Correct Answer

SCENARIO 1
You take your blood pressure medication every morning at 8:00 AM.
It is now 10:00 AM and you just realized you forgot to take it.
What is the safest thing to do?

STEP-BY-STEP GUIDE FOR THE HELPER

- 1

Read Scenario 1 aloud

Do: Read the scenario slowly. Pause. Ask the patient what comes to mind first.

Say: "Listen to this situation. Imagine it's happening to you right now."

Tip: Pause after reading. Ask: 'What's the first thing that comes to mind?'
- 2

Present options for Scenario 1

Do: Read each option one at a time. Let the patient decide.

Say: "Here are your choices. Take your time."

Tip: Cover other options while reading each one if attention is limited.

3

Discuss the answer

Do: After they choose, ask why before confirming.

Say: "Why did you pick that one? What could go wrong with the other choices?"

Tip: Discussion matters as much as the right answer. Reasoning transfers to real situations.

4

Read Scenario 2 aloud

Do: Read the second scenario clearly. Repeat the same process.

Say: "Here's a second situation. Same idea — what is the safest thing to do?"

Tip: Scenario 2 tests the double-dose temptation directly. Watch for this.

5

Reinforce the universal rule

Do: After both scenarios, state the rule clearly.

Say: "The rule is: when you miss a dose and you're not sure what to do — CALL first. Never double up and never just guess."

Tip: Ask: 'Who would you call?' Make sure they can name a specific contact.

ANSWER KEY

Scenario	Correct Action	Why
Scenario 1 — 2 hours late	Call the pharmacist before deciding	Some medications can safely be taken late; others cannot. Only a pharmacist can advise.
Scenario 2 — Next morning after missing evening dose	Call pharmacist or prescriber before deciding	Double-dosing is almost never safe and can cause overdose or dangerous effects.

WHAT TO WATCH FOR

- Patient chooses double-dosing in either scenario — the most common dangerous choice
- Patient says 'I'd just skip it' without checking — also potentially harmful for some medications
- Patient is unsure and would rather guess than call — reinforce that calling is always the right move

IF THEY STRUGGLE

- Double-dosing seems reasonable: "What does your body feel when it suddenly gets twice the medication? Is that always safe?"
- Would rather not call: "The pharmacist is there exactly for this question. It takes two minutes."
- Would skip without checking: "For some medications, skipping is fine. For others, it's not. How do you know which one this is?"

✓ IF THEY SUCCEED

Say: "The rule is simple — when in doubt, call. One phone call protects you from both under-dosing and over-dosing. That is a skill worth having." Move on to Activity 10.

SECTION 4

Complex Real-World Simulation

Activities 10 – 12

ACTIVITY

10

COMPLEX LEVEL · Cognitive Rehabilitation

Medication Management with Distractions

Overview

The patient fills a weekly pill organizer while real-world interruptions occur — a pharmacy notification, a ticking appointment clock, similar-looking bottles, and a nearly empty medication. This is the first activity that directly simulates home conditions. The goal is not just accuracy, but observing how the patient manages attention, recovers from interruptions, and monitors their own errors.

WHAT YOU WILL NEED

- App open to Activity 10 (built-in distractors) — or the physical setup described below
- A timer or phone for the appointment alarm
- Two bottles with similar-looking labels
- One bottle with only 3 pills remaining (marked)
- About 25 to 35 minutes

DISTRACTORS — HOW TO SET THEM UP FOR IN-PERSON SESSIONS

Distractor	How to Create It	What to Observe
Phone notification	Send a fake text: 'New message from pharmacy'	Does the patient stop carefully and find their place again?
Appointment alarm	Set a timer to go off 8 minutes in	Does time pressure cause rushing and errors?
Similar-looking bottles	Use two bottles with same color caps or labels	Does the patient read labels or guess by appearance?
Nearly empty bottle	Mark one bottle with '3 pills left'	Does the patient notice the refill need while working?
'Take with food' instruction	Add a handwritten label to one bottle	Does the patient notice and note special instructions?

STEP-BY-STEP GUIDE FOR THE HELPER

1

Set up and give the task

Do: Tell the patient to fill a full weekly pillbox. Do not mention the distractors in advance.

Say: "Fill the complete week using the medications on the table. Take your time and read every label."

Tip: Observe from a slight distance. Take notes. Do not intervene unless safety is at risk.

2

Let them work — observe quietly

Do: Do not help. Allow all interruptions to occur naturally.

Say: *"(Stay quiet — just observe.)"*

Tip: Note: Do they check labels? Do they self-correct? Do they stop carefully when interrupted, or lose their place?

3

Trigger the pharmacy notification

Do: At the 5-minute mark, introduce the pharmacy notification.

Say: *"(Deliver note or activate app notification quietly.)"*

Tip: Watch: Do they put the task down carefully, handle the interruption, and return to exactly where they left off?

4

Trigger the appointment alarm

Do: At the 8-minute mark, let the alarm sound without comment.

Say: *"(Let the alarm sound. Say nothing.)"*

Tip: Count errors that occur in the two minutes after the alarm. Rushing-induced errors are very common here.

5

Check for refill awareness

Do: After the task is complete, ask if they noticed anything about the bottles.

Say: *"Before we check results — did you notice anything about any of the bottles?"*

Tip: If they missed the nearly empty bottle, show it now and ask: 'What would have happened if you did not notice this until Wednesday?'

6

Review compartment by compartment

Do: Go through every compartment using the answer key from Activity 8 or the session's medication list.

Say: *"Let's check your work together. Tell me what you placed and why for each one."*

Tip: Score: accuracy, self-corrections, label use, refill awareness, interruption recovery.

ANSWER KEY

What to Score	What It Tells You Clinically
Correct compartments	Overall accuracy under cognitive load and real-world conditions
Self-corrections before submitting	Error monitoring, self-awareness, metacognition
Label reading vs. color guessing	Attention to detail versus impulsive shortcuts
Refill identification	Proactive safety awareness and prospective memory
Recovery after interruptions	Task persistence, working memory, executive function

WHAT TO WATCH FOR

- Patient loses their place after interruption and restarts from the beginning
- Errors cluster in the last few compartments after the appointment alarm (rushing)
- Patient fills by color and misses the 'take with food' instruction entirely
- Patient never identifies the nearly empty bottle

IF THEY STRUGGLE

- Loses place after interruption: Teach a 'mark and return' habit — point to where you stopped before handling any interruption
- Rushing after alarm: 'Let's pause — a missed pill is more important than being a few minutes late'
- Misses empty bottle: 'What would have happened if you ran out of this medication mid-week?'

✓ IF THEY SUCCEED

Say: "Managing medications when life is happening around you is genuinely hard. You showed us your strengths and where we can practice more. That is exactly what this activity is for." Move on to Activity 11.

ACTIVITY

11

COMPLEX LEVEL · Cognitive Rehabilitation

Hospital Discharge Medication Reconciliation

Overview

The patient compares three documents — their pre-hospital medication list, the hospital discharge instructions, and the contents of their pharmacy bag — to determine what to take, what to stop, what is new, and what might be a safety problem in the bag. This mirrors one of the highest-risk moments in patient care: the transition from hospital to home.

WHAT YOU WILL NEED

- App open to Activity 11 — or printed copies of all three documents below
- A pen to mark up documents
- About 25 to 35 minutes — this is the most clinically intensive activity in the handbook

DOCUMENT 1 — PRE-HOSPITAL MEDICATION LIST

Medication	Previous Dose & Frequency
Amlodipine 5 mg	Once daily
Aspirin 81 mg	Once daily
Metformin 500 mg	Twice daily
Fish Oil 1000 mg (supplement)	Once daily

DOCUMENT 2 — HOSPITAL DISCHARGE INSTRUCTIONS

CONTINUE Amlodipine 5 mg — once daily, no change
STOP Aspirin 81 mg — do not take
CONTINUE Metformin 500 mg — twice daily, no change
START Clopidogrel 75 mg — once daily (new medication)
Supplements (Fish Oil, vitamins): Ask your doctor before restarting any.

DOCUMENT 3 — PHARMACY BAG CONTENTS

Amlodipine 5 mg · Metformin 500 mg · Clopidogrel 75 mg · Aspirin 81 mg

Note: The pharmacy bag contains Aspirin 81 mg — which the discharge instructions say to STOP. This is a safety concern and must be identified by the patient.

STEP-BY-STEP GUIDE FOR THE HELPER

1 Read all three documents together

Do: Read each document aloud with the patient. Lay all three side by side if using paper.

Say: "Before the hospital, you were taking these medications. When you left, the hospital gave you these instructions. And this is what came in your pharmacy bag."

Tip: Read Document 2 slowly — it has five distinct instructions that must each be processed separately.

2 Identify what to continue

Do: Ask the patient which medications from the old list the hospital said to continue.

Say: "Look at the discharge instructions. Which medications from your old list does it say to keep taking?"

Tip: Correct: Amlodipine and Metformin. Watch for Aspirin being included by mistake.

3 Identify what to stop

Do: Ask the patient which medication must not be taken anymore.

Say: "Which medication did the hospital say to STOP? Find it on the discharge paper."

Tip: Aspirin. Then ask: 'Now look at your pharmacy bag — is Aspirin in there?'

4 Identify the new medication

Do: Ask the patient which medication is brand new.

Say: "Is there a medication in the discharge instructions that was NOT on your old list?"

Tip: Clopidogrel. Confirm the patient understands this must be started immediately.

5 Handle the pharmacy bag safety issue

Do: Go through the bag contents one by one. Ask the patient if each item is okay to take.

Say: "Let's check each item in the bag against the discharge instructions. Is each one okay to take?"

Tip: When they reach Aspirin: 'Should this be in here? What does the discharge paper say about Aspirin?'

6 Discuss the supplements

Do: Ask the patient what to do about Fish Oil, which is not mentioned in either column.

Say: "What about Fish Oil? It's not on the 'continue' list — and not on the 'stop' list either. What does the discharge paper say?"

Tip: Correct: hold it and call the doctor. Supplements can interact with new medications like Clopidogrel.

ANSWER KEY

Medication	Correct Decision	Reason
Amlodipine 5 mg	Continue	Discharge instructions say continue
Metformin 500 mg	Continue	Discharge instructions say continue

Clopidogrel 75 mg	Start immediately	New medication ordered at discharge
Aspirin 81 mg	Do NOT take	Explicitly stopped — bag copy is a safety concern
Fish Oil	Hold — call doctor	Instructions say ask before restarting supplements

WHAT TO WATCH FOR

- Patient plans to take Aspirin because it was in the pharmacy bag — this is a real safety risk
- Patient forgets to start Clopidogrel because it was not on the original list
- Patient restarts Fish Oil without checking — it can interact with Clopidogrel
- Patient feels overwhelmed — take a break and work through one document at a time

IF THEY STRUGGLE

- Taking Aspirin: "Check the bag against the discharge paper. What does the paper say about Aspirin?"
- Forgetting Clopidogrel: "Is there anything on the discharge paper that was NOT on your original list?"
- Restarting Fish Oil: "The discharge paper says to ask your doctor first. Why might that be important with a new blood thinner?"

✓ IF THEY SUCCEED

Say: "You just did something that many patients and caregivers find extremely difficult. Catching the Aspirin in the pharmacy bag is exactly the kind of error that causes real harm at home. You found it. That is an important skill." Move on to Activity 12.

ACTIVITY

12

COMPLEX LEVEL · Cognitive Rehabilitation

Full Medication Management Simulation

Overview

This is the capstone activity. The patient manages a realistic multi-part scenario: a dosage change, a near-empty bottle, a missed dose, a message from family, and preparation for a doctor's appointment — all at once. It integrates every skill practiced in Activities 1 through 11 and provides a direct measure of discharge readiness and independent living capability.

WHAT YOU WILL NEED

- App open to Activity 12 — or this printed page plus a blank weekly pill organizer
- The updated medication list from Activity 6 (or a new one provided below)
- Paper and pen for writing notes and responses
- About 30 to 45 minutes — rest breaks are appropriate

SEVEN TASKS — COMPLETE THEM IN ORDER

Task	What the Patient Must Do
1	Update the medication list to reflect the change from once daily to twice daily with meals
2	Fill the complete weekly pillbox using the updated schedule
3	Identify which medication has only 3 pills remaining and name it
4	Write a reminder or set an alarm to call the pharmacy before running out
5	Decide what to do about last night's missed evening dose
6	Write a short, accurate message back to their daughter about the morning pills
7	Write one clear question to ask the doctor at Friday's appointment

THE FULL SCENARIO — READ THIS TO THE PATIENT

1. You have a doctor appointment on Friday.
2. One medication has only 3 pills left.
3. Your doctor changed one medication from once daily to TWICE daily with meals.
4. Your daughter sent you a message: 'Did you take your morning pills?'
5. You think you may have forgotten last night's evening dose.

PREVIOUS DIRECTION

NEW DIRECTION (FROM DOCTOR)

Take 1 tablet once daily	Take 1 tablet TWICE daily — with meals
--------------------------	--

STEP-BY-STEP GUIDE FOR THE HELPER

1

Read the scenario — ask what needs to happen

Do: Read the scenario aloud. Ask the patient to name all five things they need to manage.

Say: *"Before we start — tell me the five things happening in this scenario."*

Tip: If they cannot recall all five, re-read and let them take written notes.

2

Task 1 — Update the medication list

Do: Ask the patient to find the medication the doctor changed and update the list.

Say: *"Which medication did the doctor change? Update it on your list. What exactly changed — the amount or how often?"*

Tip: Answer: how often — now twice daily with meals. Watch for patients who change the dose amount instead of the frequency.

3

Tasks 2 through 4 — Pillbox and refill

Do: Fill the full week using the updated list. Then identify the low-supply medication and create a reminder.

Say: *"Fill the pillbox for the whole week. Then — which medication has only 3 pills? How many days is that? What reminder will you set?"*

Tip: Have them physically write the reminder or set an alarm. Verbal-only is not sufficient for carryover.

4

Task 5 — Missed dose decision

Do: Ask the patient what they will do about last night's missed evening dose.

Say: *"You may have missed your evening dose last night. Walk me through what you would do."*

Tip: Correct answer: call pharmacist or prescriber before deciding. Double-dosing today is not safe.

5

Tasks 6 and 7 — Communication and appointment prep

Do: Ask the patient to write a message back to their daughter and one question for the doctor.

Say: *"Write a message back to your daughter. Then write one question you want to ask the doctor on Friday."*

Tip: A good doctor question: 'What should I do if I'm not home for a meal and can't take it with food?'

ANSWER KEY

Task	Full Credit	Common Error
1 — List update	Twice daily with meals, correct medication updated	Wrong medication changed, or frequency not updated
2 — Pillbox	All compartments correct; twice-daily med in both AM and PM	Twice-daily still filled once per day

3 & 4 — Refill	Correct bottle named; reminder created	Bottle identified verbally but no reminder set
5 — Missed dose	Plans to call pharmacist/provider before deciding	Plans to double-dose, or skips without checking
6 — Family message	Accurate, calm, specific reply	Vague response: 'I'm fine'
7 — Doctor question	Clear, relevant question about the new schedule	No question written, or overly general

WHAT TO WATCH FOR

- Twice-daily change missed entirely — patient fills pillbox the old way
- Refill not flagged or reminder not created even after identifying the low supply
- Missed dose plan is to take two doses today
- Family message is vague or inaccurate instead of addressing the actual question

IF THEY STRUGGLE

- Twice-daily missed: "Look at the new direction. How many times a day? How does that change your pillbox?"
- No refill reminder: "You said 3 pills — how many days is that? What might happen if you forget to call?"
- Double-dose plan: "What is the rule when you are unsure about a missed dose?"

✓ IF THEY SUCCEED

Say: "You just completed the most advanced activity in this handbook. Every skill from Activities 1 through 11 came together in this one. You are building the tools to manage your health safely at home — and that matters."

QUICK REFERENCE
Therapist Cheat Sheet

Use this page as a quick reference during any session. Key rules, common prompts, and universal coaching guidance at a glance.

Universal Safety Rules — Reinforce These in Every Session

Rule	Why It Matters
Always read the label before placing or taking any pill.	Pill color alone does not identify a medication safely.
When two instructions conflict — STOP and call before taking anything.	Medication errors at care transitions are a leading cause of preventable harm.
Never double-dose to make up for a missed dose.	Double-dosing can cause overdose, dangerous side effects, or drug interactions.
When in doubt, call the pharmacist or prescribing doctor.	The pharmacist is free, fast, and specifically trained for these questions.
Never change a dose amount or frequency without provider approval.	Self-adjusting a prescription can be as dangerous as skipping it entirely.

Universal Coaching Prompts — Use Any Time

Situation	What to Say
Patient guesses without reading	"Let's check the label first — what does it say?"
Patient gets something wrong	"Let's double-check that one together."
Patient finishes too quickly	"Before we submit — let's go through each one out loud."
Patient seems frustrated	"This is genuinely hard. Let's slow down and do just one at a time."
Patient gets it right	"Exactly right. Tell me why — so we know you've really got it."
Patient is unsure about a dose	"That is exactly the right instinct — when unsure, call first."

Medication Timing Reference

Common Instruction	Means
Once daily	1 dose every day — same time preferred
Twice daily	2 doses per day — typically morning and evening (12 hours apart)
Every 12 hours	Same as twice daily — must appear in BOTH morning and evening

Every 8 hours	3 doses per day — typically morning, afternoon, and evening
With food	Take the dose during or immediately after a meal — not on an empty stomach
At bedtime	Just before sleep — not the same as 'evening'
PRN / as needed	Only when symptoms occur — not on a fixed schedule

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